Ireland:
The Politics of Drug Policy

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The last 50 years of Ireland's drug policy reflect the competing political ideologies, institutional resistance, and crises that shaped it. From its inception, Irish drug policy took after a law and order framework, with strict criminalization, prioritizing a criminal justice approach rather than a public health approach. Historically, policymakers' reluctance to acknowledge socio-economic factors in drug addiction led to criminalization and abstinence programs. By the late 1990s, Ireland began adopting harm reduction and community-based healthcare initiatives, however it still reinforced the established punitive approaches to drug use. Over the last 20 years, Ireland has implemented more progressive harm reduction and community-based drug policies, but the criminalization approach embedded in Irish institutions and public perception resists these changes.

Irish drug policy reflects the debate between punitive, criminal justice approaches and harm reduction, community-based approaches to address drug addiction. The criminalization approach failed to meaningfully address drug addiction as a socioeconomic issue, seeing addiction as a result of individual choice rather than a public health issue fueled by poverty. Due to the failure of criminalization to reduce drug crises, harm reduction strategies have not only gained traction but have also demonstrably improved health outcomes for people who use drugs. However, the key challenge for harm reduction policies is resistance to changing approaches from both Irish institutions and public opinion. The debate and resistance to new drug policy approaches have resulted in periods of inaction and stagnation, but slowly incremental policy changes have shown that harm reduction is not just a progressive theory, but a practical and proven strategy to reduce drug use and improve public health outcomes in Ireland.

Scholars and policy analysts have debated the cause and consequences of Ireland's drug policy. Early work, such as Butler (1991) emphasized that policymakers ignored structural and

socioeconomic causes of drug addiction, like poverty, and housing inequality, and instead viewed drug addiction as a moral failure. Additionally, Butler argues that the first introduction of harm reduction policies was not a result of progressive policy making, but responses to crises and community-based activism. Later research from the European Monitoring Centre for Drugs and Drug Addiction has documented the development of drug policy, showing slow progress towards health based drug approaches through the national drug strategies. McCann and Duffin, in their study of supervised injection facility advocacy, found that reforms faced significant delays because of institutional resistance and political risk aversion.

Policy evaluations also support these critiques. The 2022 Progress in the Penal System report found little advancement in prison-based treatment services. The midterm review of the 2017–2025 National Drugs Strategy had similar findings, that nearly half of the policies in the strategy were incomplete or delayed significantly. These sources highlight that Ireland's political institutions have struggled to deliver consistent evidence-based drug policy. These reports and scholars frame Ireland's drug policy as not only a matter of reform, but a matter of colonial legacies, political constraints, and conflicting ideologies about drug use and addiction.

Early Irish drug policies reflected British colonial laws, with the Dangerous Drugs Act of 1934, that established a punitive approach towards drug use (EMCDDA, 2013). Drug policy in Ireland first shifted towards addressing drug abuse as a societal concern in the 70s with the 1971 Report of the Working Party on Drug Abuse. This report framed drug abuse as a health issue, but failed to define drug abuse clearly. The report recognized that drug users require medical attention; however, it did not challenge the exclusive focus on abstinence and supply control strategies. The first set of drug regulations following this report was the Misuse of Drugs Act 1977, that defined penalties for drug production, possession, and supply. Policy makers at the

time did not explore alternative approaches to drug policy used elsewhere, like decriminalization and harm reduction strategies, nor did they consider structural causes of drug problems like poverty, unemployment, and urban decay (Butler, 1991).

Beginning in the 1980s, heroin use dramatically increased for working-class people and unemployed youth in Dublin, exposing the failures of previous drug policies. During this time, Ireland experienced political instability, with three general elections between 1981 and 1982 and four different Ministers for Health between 1979 and 1982, which slowed down policy responses to crises. As a result, the Department of Justice and public health agencies were slow to recognize the HIV/AIDS crisis, leading to the rise of community-based activism. The 1984 Misuse of Drugs Act attempted to address these problems through harsher penalties for drug offences (EMCDDA, 2013). In response to continued rise in intravenous drug use and the onset of the HIV/AIDS crisis, the Irish government reluctantly shifted drug policy to embrace more harm reduction measures, including methadone maintenance and needle exchange programs (Butler & Mayock, 2005).

In 1991, the Irish government formalized the policy shift by introducing the first national drugs strategy, acknowledging the need to balance criminal justice supply reduction with healthcare demand reduction. This national drug strategy also acknowledged the role of environmental factors in drug use. However, policy makers lacked concrete plans for achieving policy goals, and institutions resisted new strategies, resulting in very slow structural change. By 1996, the Ministerial Task Force on Drugs formally recognized the link between poverty, social exclusion and drug addiction. During this time, in the most affected areas, local and regional drug task forces started working with community-based methods (EMCDDA, 2013). Despite reforms and policy shifts, the Criminal Justice Act of 1999 reinforced the criminalization

approach by introducing mandatory minimum sentences for drug offenses (McCann & Duffin, 2023).

By the 2000s, Ireland had one of the highest drug related death rates in Europe, a result of continued criminalization despite efforts towards harm reduction (Irish Examiner, 2024). Limited service scope hindered the implementation of harm reduction policies; for example, although methadone programs continued to expand, access remained limited outside Dublin, creating regional barriers for drug users. The 2001-2008 National Drugs Strategy was one of the largest shifts towards public health oriented policies, emphasizing harm reduction, treatment, and rehabilitation, but still embraced law enforcement as an approach to drug use. Despite the expansion of harm reduction programs, policymakers continued to support law enforcement crackdowns as solutions to new problems. The 2010 Criminal Justice (Psychoactive Substances) Act tried to combat new synthetic drugs but ultimately reinforced prohibitionist policies instead of public health approaches (EMCDDA, 2013).

By the 2010s, public and political attitudes shifted largely towards decriminalization and health-based approaches to drug use. This transition was influenced by growing awareness of the failures of punitive drug laws, and the successes of international models like Portugal, that demonstrated the effectiveness of harm reduction. The 2017–2025 National Drugs Strategy ("Reducing Harm, Supporting Recovery") is the most progressive drug policy in Ireland to date, focusing explicitly on not only harm reduction, but social reintegration, and other public health measures. Unlike earlier strategies that focused on supply control and law enforcement, the policy focuses on promoting harm reduction, supporting recovery and rehabilitation, reducing drug-related harm, and improving interagency coordination.

The midterm review of the National Drugs strategy in 2021, assessing progress from 2017 to 2020, highlighted that there is a continued gap between policy and practice. Out of the 50 actions outlined in the initial strategy, the report found that only 25 were completed or on track, and 20 were progressing with issues. While many health-based policies expanded, like treatment centers, limited funding and resources were a significant barrier to the full implementation of harm reduction services. Additionally, despite the focus on harm reduction strategies for drug use, drug possession for personal use remained a criminal offense, which undermined the effectiveness of new services. The report shows that for Ireland to fully transition from punitive to health-based approaches towards drug use, there must be the political will to change the existing framework for criminalization and to allocate the proper resources for new initiatives.

The 2022 Progress in the Penal System report by the Irish Penal Reform Trust highlights the lack of changes to drug treatment services in prison and the need for updated data and policy reforms to effectively address addiction within the incarcerated population. Their assessment of prison conditions, healthcare services, family contact, rehabilitation programs, and accountability measures found that only 4 out of 28 standards progressed. The existing Irish Prison Service drugs policy, *Keeping Drugs out of Prison* was created in 2006 and not only requires an update, but new data on the number of prisoners dealing with addiction issues to effectively keep up with the needs of the incarcerated population. The report advocates for alternatives to imprisonment that include access to treatment as the penal system is inefficient at dealing with drug use.

The biggest challenges to achieving progressive policy goals is the lack of funding and political support necessary for implementation. The 2015 Misuse of Drugs (Amendment) Act that introduced the framework for medically supervised injection facilities suffered from those

challenges. To push support for SIFs, Irish harm reduction advocates used international evidence to show that SIFs reduce harm without increasing crime. For example, Vancouver's Insite reported a 35% decrease in overdose deaths in the surrounding area, and Sydney's MSIC supervised over one million injections without a single on-site death(McCann & Duffin, 2023). Eventually, after overcoming institutional barriers, law enforcement opposition, and local political resistance, the first official SIF opened in 2024.

The 18 month pilot SIF program opened in December 2024 at Merchants Quay Ireland's Riverbank Centre in Dublin. As of January, the facility already had more than 300 visits by a total of 108 people (Nally 2025). By February, the center staff managed 15 overdoses, with no fatalities, primarily administering oxygen (Wilson 2025). The usage of the facility and its successful management of overdoses without fatalities illustrates the lifesaving potential of harm reduction in practice, something that punitive models cannot achieve. By providing a safe stigma free environment, the SIF connects drug users in Dublin to the healthcare system, meeting them where they are and responding to their needs.

Despite the successes of harm reduction, the 2024 election highlighted the political debate surrounding decriminalization and harm reduction. The country's two dominant centrist parties, Fianna Fail and Fine Gael, have historically embraced criminalizations approaches with hesitance to expand harm reduction. While both parties have recognized the need for drug policy change, their political actions suggest that their caution regarding progressive reforms has slowed down the implementation of progressive drug policy. The Labour party on the other hand has been a consistent advocate for health-based drug use approaches, and have often led charges for progressive drug legislation.

Fianna Fail has increasingly presented itself as supportive of health-based harm reduction approaches. Ahead of the 2024 election, they published policy statements in favor of decriminalizing drug possession for personal use, which was a shift from its historically punitive approach. However, they later limited this policy to only cannabis, excluding stigmatized substances like heroin (McTaggart 2024). Their policies and actions are equally unclear, which limits effective drug legislation despite progressive aims, because of vague language.

Fine Gael continues to favor criminal justice responses over public health responses. The party continues to resist decriminalization, especially of cannabis and opioids. While Fine Gael is not openly against harm reduction approaches, they reinforce a policing and public order narrative that stigmatizes drug use and limits the impact of harm reduction (MacRedmond, 2024). Fine Gael presided over the 2015 legislation that created the basis for SIF programs, but the delay of their implementation shows the party's aversion to taking political risks, wanting to appear receptive to public health needs without provoking backlash. Their cautious approach reflects their law and order base and their concerns about alienating middle-class voters.

The Labour party, unlike Fianna Fáil and Fine Gael, has maintained a clear stance in favor of decriminalization and harm reduction. They have consistently approached drug addiction as a public health crisis, instead of a criminal issue. Ahead of the 2024 election, the Labour platform included the decriminalization of possession of drugs. For personal use a new national drugs strategy, and acting on recommendations made during the interim report (McTaggart 2024). Labour has also consistently called out Fianna Fail and Fine Gail for their inconsistencies and outdated drug policy approach, positioning itself as the only major party fully aligned with evidence-based drug reform (O'Connell 2023).

The political debate surrounding drug policy in Ireland reflects the divide over how addiction should be addressed and the understanding of what factors cause addiction. While Labour and other progressive parties and independents continue to support decriminalization and health-based approaches to drug use, Fianna Fáil and Fine Gael continue to fall short of supporting these policies fully, acknowledging the need for reform while simultaneously slowing down implementing it. While public discourse has swayed towards progressive policies, there still remains the associations between drug use with crime and moral failure, which makes it a political risk for centrist parties to fully embrace progressive reform. As a result, drug policy has developed inconsistently, shaped more by politics than public health needs.

Ireland's approach to drug policy throughout its history has been a debate between the traditional and overly punitive methods of criminal justice and the emerging, health-centered methods of harm reductions. While recent policies, like the 2017–2025 National Drugs Strategy, indicate a shift towards community and healthcare based responses, institutional resistance, political caution, and lack of resources have impeded the implementation and effectiveness of these policies. The success of the 2024 SIF in Dublin proves that harm reduction works in Ireland, saving lives, reducing community harm, and offering access to healthcare for marginalized populations.

Significant political divisions among policymakers and an abundance of political caution considerably slow the reform process, creating a significant obstacle to progress. Fianna Fáil and Fine Gael, while supporting health based rhetoric, have largely reinforced the existing punitive framework for drug use. Fianna Fáil's 2024 pledge to decriminalize drug possession that they later narrowed to just cannabis reflects the party's cautious approach to progressive policy. Fine Gael, despite backing small specific programs, continues to emphasize law and order narratives

that reflect their voter base. Both parties prioritize avoiding political risks over evidence-based policy, preventing substantial change.

On the other hand, Labour, along with other progressive parties and independents, has shown consistent support for reform. Labour's policy commitments to decriminalization, investment in treatment, and the development of a new national drug strategy illustrate the political forces necessary for drug policy change. However, their political opposition and limited power prevent the party from single-handedly transforming Irish drug policy. Public discourse mirrors the political discourse, shaped by media sensationalism, community resistance, and ingrained stigmas that reinforce older models of drug policy.

Ireland's drug policy experience underlines that implementing harm reduction is not just a policy issue, but a political one. The consistent failure to fully implement health based drug approach is not due to a lack of evidence or public need, but rather it comes from the risk avoidance of parties unwilling to dismantle colonial remnants of punitive drug approaches. While recent strategies have adopted progressive harm reduction rhetoric, actual institutional change has been slow. Until there is not only a rhetorical consensus, but a continued legislative desire to change and improve the response to drug use in Ireland, harm reduction will remain limited to smaller programs rather than a fully realized healthcare approach. The future of Irish drug policy relies on confronting challenges, framing addiction as a health issue rather than moral failure, and building a strong political base that prioritizes social equity and evidence-based care. Failure to achieve reform not only harms individuals in vulnerable populations, but reflects deeper issues of inequality and social stigma in Irish society.

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